



A GUIDE FOR WOMEN ON GLP-1 MEDICATION

The Ownership Protocol

What actually happens when the shot stops doing the work for you — and the four things worth building while it still can.

Based on your
Ownership Score

Muscle · Structure
Fuel · Situations

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You took the Audit.

You got your score.

And somewhere on that page, one of four words probably stood out more than the others:

Muscle. Structure. Fuel. Situations.

Maybe one of them came back noticeably lower. Your leak.

And if you're being honest, there was probably a moment when you looked at that result and thought:

Yeah. I knew that.

Not necessarily because you'd been doing anything wrong. Sometimes the things we know are true are simply the things we've gotten very good at not looking at too closely.

That's what the Audit is for.

This isn't another generic "stay healthy on a GLP-1" pamphlet. You aren't a generic patient with a generic problem. You have a specific result, a specific starting point, and one place where your current result is more vulnerable than the others.

This guide is about what that score actually means—and what to do next.

Because there's a bigger question underneath the number on the scale. You can probably feel it.

The weight is coming off. Maybe it's coming off faster than you ever thought possible. Clothes fit differently. People are noticing. You're getting the texts, the compliments, the double takes.

And underneath all of that relief, there's a quieter thought that tends to show up when the room gets quiet:

Okay. But what happens later?

Maybe you've seen the stories. The woman who hit a wall around month four. The friend who stopped taking the medication and slowly watched the weight come back. The headlines about muscle loss that you filed away for later because, honestly, the scale was finally moving and you weren't about to ruin a good thing by worrying about something that hadn't happened yet.

Fair enough. But here's the part nobody really sits you down and explains at the beginning:

The medication was never the whole plan.

It solved something enormously important. It quieted the appetite. It turned down the food noise. It made eating less possible without requiring you to spend every waking hour negotiating with hunger. That's a big deal.

But it doesn't build muscle for you. It doesn't teach you how to structure a day of eating. It doesn't automatically teach you what makes a meal satisfying and nutritionally useful. And it doesn't rehearse what you'll do when you're stressed, traveling, celebrating, exhausted, or having one of those weeks where everything seems to go sideways at once.

Those things still have to be built. Not because you shouldn't have taken the medication. Not because the medication isn't working. And definitely not because you need to "prove" you can do this without help. Quite the opposite.

The medication gives you something incredibly valuable: breathing room.

The question is what you build in that space. That's what this guide is about.

Not stopping the medication. Not fighting it. Not pretending you made the wrong decision by starting.

Just using the window you've been given to build the part that has to belong to you.

01

PART ONE

The Problem

Same medication, same starting point — two very different endings. This is where the split happens, and why.

| Chapter 1 · The Two Endgames

| Chapter 2 · What the Medication Actually Does

The Two Endgames

Let's fast-forward for a moment. Imagine a date on the calendar somewhere in the future.

You don't know exactly when it will be. Maybe your dose changes. Maybe insurance changes. Maybe side effects become a reason to stop. Maybe you're planning for pregnancy. Maybe you've simply reached a point where the medication isn't doing what it used to.

Or maybe nothing dramatic happens at all. The appetite suppression just gets quieter. The effect that once felt almost miraculous becomes... normal.

Whatever the reason, for almost every woman taking a GLP-1, there is eventually a point where the medication is doing less—or doing nothing. And that's where the story splits.

ENDGAME ONE

The appetite suppression fades. The old hunger comes back. And underneath it, waiting patiently, are the same habits that were there before.

They weren't fixed. They were simply easier to ignore because the medication had turned down the volume.

So portions get bigger. Cravings get louder. Stress starts finding the same old exits. The scale begins creeping upward. Not necessarily all the weight—not necessarily overnight. But enough. Enough to create that awful feeling that the last year is quietly being erased.

And perhaps the most frustrating part? She didn't feel like she failed. She did what she was supposed to do. The medication worked. She lost the weight. The scale just stopped cooperating when the medication stopped doing as much of the work.

ENDGAME TWO

The appetite suppression fades. And... not much happens.

She still eats differently. She still has enough muscle to support the body she's built. She knows how to assemble meals without needing a prescription to tell her when to stop.

She has routines. She has tested responses for stressful weeks, travel, restaurants, holidays, and the other little ambushes life likes to throw at people.

The medication was part of the solution. But it wasn't holding the entire solution together.

Same medication. Same basic starting problem. Two very different endings.

The difference isn't willpower. It's not that one woman is disciplined and the other isn't. And it certainly isn't that one woman was "good enough" to deserve her result.

The difference is what got built while the medication was doing its part. That's the piece that matters.

And I want to be especially clear about something here:

Using a GLP-1 isn't a character flaw.

It isn't cheating. It isn't taking the easy way out. For many women, it is the first intervention that has made meaningful weight loss possible after years of trying everything else.

So don't turn this into a morality story. You didn't do something wrong by taking the shot. The problem is simply that very few people explain what needs to happen alongside it.

The medication can quiet the symptom. You still have to build what remains when the volume comes back up.

What the Medication Actually Does

You don't need a pharmacology degree for this. You need one useful mental picture.

A GLP-1 medication does one thing extremely well:

It turns down appetite.

It slows stomach emptying. It changes signaling between the gut and brain. It reduces what many people describe as "food noise"—that persistent background hum of thinking about food, planning food, wanting food, negotiating with food.

For some people, that noise has been there for so long that its absence feels almost surreal. Suddenly you can eat a normal meal and move on. Suddenly you aren't thinking about what you're going to eat next while you're still eating. Suddenly the thing that used to require an exhausting amount of mental energy is... quieter.

That's powerful. And it's one reason these medications can succeed where years of "just use more willpower" advice didn't.

But there's a distinction you need to understand.

Appetite suppression is rented.

It works beautifully while you're paying for it. But rented things are still rented. The moment you stop paying, you don't get to keep them.

The early weight loss is real. The transformation is real. The relief is real. But if the only thing holding the result together is a medication suppressing appetite, then the result is dependent on something outside you. That's the **Rented** part.

Owned is different. Owned means the things supporting your result live in your body and your life. They don't disappear because a prescription changes. They don't require a monthly payment. They don't need a particular dose to exist. They've become part of how you operate.

There are four of these assets. And you've already met them.

1 • MUSCLE

When appetite drops, food intake often drops with it. If protein and resistance training aren't deliberately maintained, some of the weight you lose can be lean mass as well as fat.

That matters for more than appearance. Muscle contributes to your metabolic floor—part of the machinery determining how much energy your body uses simply to keep going.

Lose muscle quietly while the number on the scale is celebrating, and you may be lowering the floor at exactly the moment you'll eventually need that floor to help support your result.

2 • STRUCTURE

Here's a strange side effect of having less appetite: you may never have to practice managing appetite. There might be no 3 p.m. craving to negotiate with. No overwhelming urge to snack. No giant internal debate about whether you're actually hungry.

That's wonderful while it lasts. But it also means you may not have developed much structure around eating because, frankly, you didn't need to.

A skill you never had to practice is still a skill you haven't built. That's not a criticism. It's just how skills work.

3 • FUEL

Eating less and eating well are not the same thing. You can lose forty pounds while still having no idea how to build a meal that genuinely supports you.

Enough protein. Enough real food. Enough nourishment. Food that keeps you satisfied and functioning instead of simply being whatever didn't feel like "too much."

When appetite is quiet, nutritional gaps can hide surprisingly well. Right now, the gap may not seem expensive. Later, it can be.

4 · SITUATIONS

Stress. Travel. Restaurants. Holidays. A brutal week at work. A fight with someone you love. The kind of Tuesday where everything goes wrong before 10 a.m.

These are the moments when old patterns used to take over. But if the medication has quieted the appetite driving those patterns, you may not have had many opportunities to practice a different response.

The skill didn't disappear. It may simply never have been built.

Put all four together and you get the bigger picture:

The medication can mute the symptom without building the assets that determine whether your result can stand when the symptom gets louder again.

That's not a secret defect in GLP-1s. It's simply outside the job description.

And your Ownership Score is telling you where you stand today—while you still have the advantage of building these assets with the medication helping you.

That's a much nicer way to find out than discovering the answer after the medication is gone.

02

PART TWO

The Science

A simple way to picture what's holding your result up — and why building ownership means working with the medication, not against it.

| Chapter 3 · Owned vs. Rented

| Chapter 4 · Why This Doesn't Mean Fighting the Medication

Owned vs. Rented

Let's make this even simpler.

Imagine your weight-loss result sitting on a table. Four legs hold the table up. One is Muscle. One is Structure. One is Fuel. One is Situations.

Now imagine some of those legs are solid wood. Those are **Owned**. Others are temporary supports. Those are **Rented**.

If you remove the rented supports and the table still stands, you've built something durable. If you remove them and everything starts wobbling, you've learned where the work is.

That's the whole idea.

At one end of the spectrum, a result is almost entirely Rented. The medication is suppressing appetite. The old eating patterns are largely unchanged. Muscle reserve may be low. Nutritional habits haven't been developed. Stress and travel still have the same old escape routes. Take away the appetite suppression and the system has very little else holding it together.

At the other end is a result that's largely Owned. There is enough muscle to support the body. Eating has structure. Food choices make sense. And the hard situations aren't theoretical anymore—you've actually practiced handling them.

Most people aren't at either extreme. Most women are somewhere in the middle. And here's where it gets interesting:

You can be Owned in one asset and Rented in another.

You might strength-train consistently but have no idea what your meals look like on a chaotic week. You might have excellent nutrition knowledge but almost no resistance-training habit. You might have strong Structure and Fuel but discover that one particular social situation still sends you straight back into old patterns.

That's why the score isn't simply "good" or "bad." It's a map.

Your Ownership Score shows how much of your current result is already supported by things you own, and where you're still relying heavily on the thing you're currently renting.

The lowest asset is your leak. And the leak matters because that's usually where pressure shows up first.

Why This Doesn't Mean Fighting the Medication

There's a tempting conclusion here:

If I want to own my result, maybe I need to get off the medication.

No. That's not the lesson.

Trying to overpower the medication defeats the point of having it. The goal isn't to make appetite harder. The goal is to use the fact that appetite is currently quieter.

Think of it this way. Suppose you've spent years trying to build a new eating routine while someone is standing beside you with a megaphone shouting, *FOOD. FOOD. FOOD.*

Then one day, someone turns the megaphone off. You don't look around and say, "Well, this is cheating. Better turn it back on." You use the quiet.

That's what this window is for.

A quieter appetite makes it easier to establish structure. It's easier to practice resistance training when you're not simultaneously fighting intense hunger. It's easier to learn what a satisfying meal looks like when food noise isn't hijacking every decision.

The medication isn't the obstacle to ownership.

It may be the best environment you'll get for building it.

Each asset has its own job.

MUSCLE

Give your body a reason to keep it. That means consistent resistance training and enough protein to support your needs.

Rapid weight loss without attention to protein and resistance training can increase the risk of losing lean mass along with fat. This is one of the most preventable parts of the typical endgame—and one of the easiest to overlook while the scale is moving in the right direction.

STRUCTURE

Practice eating patterns now. Regular meal timing. A repeatable idea of what a complete meal looks like. A few anchors that make your day less dependent on whether your appetite happens to speak up.

You're building the scaffolding before you need it.

FUEL

Use the quieter window to learn what actually feeds you well. Not simply what you can tolerate. Not just the smallest amount you can get away with.

Protein. Real food. Meals that support the body you're trying to maintain.

SITUATIONS

Practice when the stakes are low. Take the trip. Go to dinner. Handle the stressful week. Notice what happens. Try a different response. If it doesn't work, adjust.

The point is to discover what works while you still have a buffer. Because the worst possible time to discover that your strategy doesn't work is the first stressful month after the medication stops doing as much.

You don't have to wait. In fact, waiting is probably the one thing this guide doesn't recommend.

The best time to build ownership is while the medication is making the work easier.

03

PART THREE

The Protocol

Four daily basics everyone keeps, plus a four-week deep dive into whichever asset your Ownership Score flagged as your leak.

| Chapter 5 · The Base, and Your Branch

| Chapter 6 · How This Fits Into Your Actual Week

The Base, and Your Branch

Here's where we stop talking about the concept and start doing something with it.

The protocol has two layers.

The Base — four small daily actions, one for each asset. Everybody does these, starting now.

They're intentionally light. This isn't a dramatic seven-day transformation plan that requires a color-coded spreadsheet and a new personality. The Base exists to keep all four assets alive while you put extra attention on your leak.

Your Branch — the four-week deep dive into the asset your Ownership Score identified as weakest. This is where the heavier lifting happens.

If Muscle is your leak, you do the Muscle branch. If Situations is your leak, you do Situations. The other three stay at Base level.

Why? Because trying to rebuild everything simultaneously is how ambitious plans become abandoned plans.

Fix the leak first.

The other assets don't disappear. They stay maintained in the background and get their turn later.

And if two assets are essentially tied? Choose the one you'd be most relieved to have fixed. That's usually your answer.

MUSCLE

Have one protein-forward food at your first meal. Eggs. Greek yogurt. Cottage cheese. A protein shake. Leftover chicken. It doesn't need to be glamorous—it needs to contain real protein. Your appetite is quieter right now, which means the food you do eat has to pull its weight.

STRUCTURE

Eat at roughly the same few times each day. Not perfectly, not on a military schedule—just consistently enough that eating stops being completely random. Random is easy when appetite isn't giving you much information. Structure gives you another signal.

FUEL

Get at least one full-size portion of vegetables or fruit somewhere in your day. This isn't a calorie rule. It's a reminder that a smaller appetite shouldn't accidentally turn into a diet made entirely of whatever is easiest to get down.

SITUATIONS

Once a day, notice one moment when the old version of you might have reached for food. Then notice what happened. Maybe you ate, maybe you didn't, maybe you weren't even tempted—that's fine. You're not fixing the pattern yet. You're learning to see it.

That's the Base. Four actions. None dramatic. All daily. Keep them going for all four weeks, regardless of which branch you're working.

● The Muscle Branch

For readers whose Ownership Score flagged Muscle as the leak.

WEEK 1

Establish the floor

Two short resistance sessions. Twenty to thirty minutes each. Bodyweight or light weights. Full body. Don't chase exhaustion—your first job is simply to give your body a reason to keep the muscle it already has.

At the same time, bring protein into every meal rather than only your first one. You're building the signal from both directions.

WEEK 2

Add a third session

Three sessions this week. If two felt easy, add a little resistance or a few reps. If two felt difficult, stay at two. There's no trophy waiting for the person who rushed. Consistency beats heroics.

WEEK 3

Progressive load

Keep the three sessions. Now make them slightly harder—a little more resistance, a few more reps, a slightly greater challenge. The signal becomes stronger through repetition. Your body doesn't hear "please keep this tissue" once. It hears it again and again.

WEEK 4

Hold and check

Don't add anything new. Look at what you've built. Are you still completing three sessions without needing a motivational speech beforehand? Is protein at every meal becoming normal rather than an event?

If yes, you've built something worth carrying into Part 4. If it's still a battle every time, that's useful information—the answer may not be "try harder." The sessions may simply be too long or too difficult. Reduce the difficulty. Keep the frequency.

● The Structure Branch

For readers whose Ownership Score flagged Structure as the leak.

WEEK 1

Name your pattern

For one week, write down **when** you eat. Not what, not how much—just when. Don't change anything yet. You're looking at the actual shape of your day.

Most people are surprisingly bad at predicting their own eating patterns. The goal this week is curiosity, not correction.

WEEK 2

Pick three anchors

Look at week 1. Now choose three rough eating times that fit your actual life—not the imaginary version where you wake up refreshed, have forty-five minutes for breakfast, leave work exactly on time, and never get stuck in traffic. Your real life.

Choose three anchors and try to land near them most days. They're not prison bars. They're scaffolding.

WEEK 3

Build one real meal template

Choose one meal each day to make almost automatic. Breakfast is often easiest. Build a repeatable version containing:

- Protein
- A vegetable
- A starch or fat

Then vary it however you like. The point isn't boring food. The point is fewer decisions. When appetite is limited, you don't want every meal to become a creative writing exercise.

WEEK 4

Hold and check

Are the three anchors becoming automatic? Does the repeatable meal actually get repeated? Or did everything quietly drift back toward random?

Be honest. Structure is particularly good at disappearing when nobody is looking. This week is where you find out whether yours is starting to stick.

● The Fuel Branch

For readers whose Ownership Score flagged Fuel as the leak.

WEEK 1

Audit one day, honestly

Write down everything you eat on one normal day. Not your healthiest day, not Monday morning after you've decided to "get serious." A normal Tuesday.

The point is to see the difference between **eating less** and **eating well**. You can't close a gap you haven't actually looked at.

WEEK 2

Fix the biggest gap

Look at your day. Find the single biggest problem. Often it's protein. Sometimes it's simply not enough food overall. Whatever it is, fix **one thing**. Not five. One.

A single improvement you can actually keep is worth more than an impressive plan that lasts four days.

WEEK 3

Build your go-to list

Create a list of five to seven meals that are:

- Easy
- Nutritionally solid
- High enough in protein for your needs
- Made from real food
- Actually enjoyable enough that you'll eat them again

This becomes your emergency menu. For the days when appetite is quiet and decision-making feels like work, you don't have to invent dinner. You already know what works.

WEEK 4

Hold and check

Are you using the list? Or are you still grabbing whatever is easiest? Is the change from week 2 still happening, or did it disappear as soon as it stopped feeling new?

That's what you're checking. Not whether you were perfect. Whether the habit survived contact with ordinary life.

● The Situations Branch

For readers whose Ownership Score flagged Situations as the leak.

WEEK 1

Map your triggers

Think back to life before the medication. What reliably sent things sideways? Stress? Late nights? A particular kind of social event? Travel? A certain time of day?

Write down the two or three situations that mattered most. "Stress eating" is usually too vague to be useful. You're looking for the actual scene.

WEEK 2

Build one response

Choose your biggest trigger. Now decide what you'll do next time it appears. Make it specific.

Not: *"I'll be more mindful."* Instead: *"I'll take a five-minute walk before deciding."* Or: *"I'll text a friend first."*

The more concrete the plan, the easier it is to use when your brain is already busy dealing with the trigger.

WEEK 3

Test it live

Your trigger may show up naturally—if it does, use the plan. If it doesn't, create a low-stakes version. Go to the restaurant. Take the trip. Sit through the stressful call.

Practice while the medication is still giving you a little breathing room. You're not waiting for life to test you. You're rehearsing.

WEEK 4

Hold and check

Did your response work under actual pressure? If yes, congratulations—you now have something much more valuable than a good intention. You have evidence.

If it didn't work, that's not failure either. It's information. Look back at the second trigger you identified in week 1. That may be the next place to build.

How This Fits Into Your Actual Week

A protocol only works if it survives Tuesday. So let's put it into real life.

THE REGULAR WORKWEEK

Protein at breakfast happens before you leave the house. Eggs. Yogurt. A shake. Whatever you'll actually eat.

If you're on the Muscle branch, schedule resistance training where it fits naturally—before work, at lunch, or in the evening. The best time is the one you repeat.

For the daily Situations check-in, attach it to something that already happens. Your commute. Brushing your teeth. Getting into bed. One existing habit becomes the trigger for the new one.

BUSY OR TRAVEL DAYS

This is where the Base earns its keep. Everything may fall apart around you. The Base doesn't have to.

Protein at the first meal still matters. Hotel breakfast? Eggs. Yogurt. Something protein-forward. Airport? There is usually something workable if you look for it.

Missed a resistance session? Don't punish yourself by doubling the next one. Just continue.

And if travel is your Situations leak? Excellent—you've just found your training ground. A difficult travel day isn't proof the protocol failed. It's data.

SOCIAL OCCASIONS

Dinner out. Birthday parties. Holidays. The goal isn't perfect adherence. It's staying conscious.

If Situations is your branch, use the response you built in week 2. This is exactly what it was for.

If Situations isn't your branch, keep the Base. Notice the moment. Get your protein where you can. Eat the meal. Enjoy the evening.

And if one day doesn't look like the others? Let it be one day. A single off-pattern meal doesn't require an off-pattern week.

04

PART FOUR

Troubleshooting & Maintenance

*What to do when the plan meets an ordinary, imperfect week —
and what stays with you once the four weeks are over.*

| Chapter 7 · Common Problems and What Actually Fixes Them

| Chapter 8 · Life After the Four Weeks

Common Problems and What Actually Fixes Them

The protocol is designed to be doable. That doesn't mean every day will cooperate.

Your life has been doing its thing for a while. Your body has been doing its thing for a while. Habits don't always appreciate being reorganized. So when friction shows up, don't turn it into a personality assessment. Solve the actual problem.

“ I'm not hungry enough to eat protein at breakfast, let alone a full meal.

Fair. The medication is doing exactly what it's supposed to do. You don't need to force down a giant breakfast. Start small—a hard-boiled egg, a few spoonfuls of Greek yogurt, a small protein shake. Something with real protein. Fifteen grams is better than zero.

And if even that feels like too much first thing, move it slightly later to a time when your appetite is a little more available. The goal is not to win a breakfast-eating contest. It's to give your body something useful.

“ I feel too nauseous or low-energy to exercise.

Take that seriously. You don't get extra points for suffering through a workout. Make the starting point smaller—a ten-minute walk counts. And during a rough week, staying at Base level rather than adding resistance sessions can be the correct decision.

If nausea, fatigue, or another symptom feels more significant than a normal adjustment, that's a conversation for your prescriber. Not a willpower problem.

“ I don't want to build muscle. I just want to be smaller.

This one deserves a straight answer. The Muscle branch isn't about becoming visibly bigger. For most women, particularly while eating less, resistance training is about protecting lean mass and supporting the body you're working toward.

You're not trying to turn yourself into a bodybuilder. You're trying not to lose useful tissue simply because the scale is moving.

And if food or exercise ever starts feeling less like a tool and more like something you cannot stop controlling, that's outside the scope of this guide. Talk to a qualified professional directly.

“ I'm about to reduce my dose, or come off soon. Is it too late?”

No. In fact, this may be the moment when the work matters most. Starting earlier gives you more time to build the four assets. But starting later is still starting.

The protocol doesn't become useless because you're closer to a medication change. Start where you are. Build what you can.

“ I don't have time for all of this.”

Good news: this isn't supposed to become your second job. The Base is designed to require very little actual effort spread across the day. The Branch adds more, but only to one asset at a time.

If life gets particularly chaotic, keep the Base and pause the branch progression. **Hold. Don't quit.** You don't need a perfect month. You need a system that survives imperfect ones.

“ I already work out and eat well. Do I still need this?”

Probably less than most people. And your Ownership Score should reflect that. If your score is genuinely high across all four assets, you may already be close to Owned. This guide is then less of a rebuild and more of a confirmation.

But if one asset still showed up as your leak, don't ignore it simply because the rest of the system looks good. A strong house can still have one weak beam. Four weeks is a reasonable amount of time to reinforce it.

“ I keep forgetting or falling off after a good week.”

Welcome to being human. Usually, falling off isn't proof that you lack discipline. It's a clue that the plan was slightly bigger than the week could carry.

Drop back to the Base for a few days. Let things become easy again. Then restart the Branch from where you left off—not week one. You don't erase what you've already built because you had a bad Thursday. Momentum matters more than streaks.

“ My family or partner doesn't understand why I need to change anything. The weight is already coming off.”

They're looking at the same number you are. And right now, that number looks great. The future isn't visible on the scale yet.

You don't need to convince them of the entire four-asset framework. You just need them to understand that you're spending the next month building something that matters to you. That's usually enough. You can let the results explain the rest.

Life After the Four Weeks

Four weeks isn't a finish line. It's a construction period. You're taking the asset that was weakest and getting it from *thin* to *functional*.

After that, the maintenance gets smaller. Much smaller.

Three things. That's it.

1 Keep protein at your first meal.

This is one of the highest-leverage habits in the entire guide. It's simple. It's cheap. It's easy to repeat. Keep it.

2 Keep what your Branch built.

Maybe that's your resistance sessions. Maybe it's your three eating anchors. Maybe it's your go-to meal list. Maybe it's the situation response that actually worked. Keep it at whatever frequency you discovered was sustainable by week four—not the maximum frequency, the sustainable one.

3 Keep the daily Situations check-in.

Even after Situations stops being your active Branch, keep noticing. It's your early-warning system. It can catch problems in Muscle, Structure, or Fuel before they become problems you can see on the scale.

HOW YOU'LL KNOW IT'S HOLDING

- The scale stalls and you don't panic.
- You know the difference between a blip and a trend.
- You can explain, specifically, what's supporting your result besides the medication.
- You miss a week without turning it into a missing month.

That's ownership. Not perfection.

Stability.

AND IF YOU SLIP?

Go back to the Base for one week. Exactly as written. No heroic comeback. No punishment. No "starting over Monday." Just return to the four basics.

Most people find their way back within days. And the second time is usually easier because you're not starting from zero. You're returning to something you've already built.

CLOSING

You came into this with a plan that was working. And underneath that success was a question you couldn't quite shake.

"What happens when the shot isn't doing this much anymore?"

That question wasn't pessimistic. It was smart.

And the answer isn't that you should have done things differently. You didn't make a mistake. The medication did its job. It quieted the hunger. It turned down the food noise. For some women, it creates the first stretch of their adult lives where food isn't occupying a ridiculous amount of mental real estate.

That's not a small thing. That's freedom.

But freedom creates a new question:

What are you going to build with it?

That's the part this guide is about. The prescription can't build your muscle. It can't practice your eating structure. It can't teach you what foods actually fuel you well. And it can't sit beside you at the airport, at Thanksgiving, after a terrible day at work, or during the week when everything feels like too much and make the decision for you.

Those are yours. Not as a punishment. As an opportunity.

Because the goal was never to prove you could do this without medication. The goal is to make sure the medication isn't the only thing standing between you and the result you've worked so hard to create.

Four assets. One leak to start. A Base you can carry through ordinary life. Four weeks of focused work. Then three things to keep.

That's the protocol. It isn't complicated. It just needed to be explained before you needed it.

So start where your score says to start. Not where you wish you were. Not where you think you should be.

Where you are.

The four weeks begin whenever you decide they do.

REFERENCE

Quick Reference

Pull this page out and keep it somewhere visible. Everything else in this guide leads back here.

THE BASE · EVERY DAY, EVERY WEEK, REGARDLESS OF BRANCH

ASSET	DAILY ACTION
Muscle	Protein-forward food at your first meal
Structure	Eat at roughly the same few times each day
Fuel	One full portion of vegetables or fruit
Situations	Notice one moment you'd have reached for food before

YOUR BRANCH · FOUR WEEKS, FOLLOW THE ASSET YOUR SCORE FLAGGED

WEEK	MUSCLE	STRUCTURE	FUEL	SITUATIONS
1	2 short resistance sessions	Track your real eating times	Audit one honest day	Map your top 2–3 triggers
2	Add a 3rd session	Set 3 rough eating anchors	Fix the single biggest gap	Build a plan for your top trigger
3	Progressive load	Build one repeatable meal	Build a 5–7 meal go-to list	Test the plan live
4	Hold and confirm	Hold and confirm	Hold and confirm	Hold and confirm

AFTER WEEK FOUR

Three things to keep indefinitely:

1. Protein at your first meal
2. Whatever your Branch built, at whatever frequency you landed on
3. The daily Situations check-in

That's it. Three things. Not forty.

This guide is a self-assessment and lifestyle framework, not a medical diagnosis or treatment plan. It includes no medication dosing guidance of any kind. If exercise, eating, or appetite ever starts to feel compulsive or distressing rather than manageable, or if any symptom feels like more than a normal adjustment, please talk to your prescriber or another qualified professional directly.